

AUTONOMY, CAPACITY, AND THE RIGHT TO EXIT

A Clinical and Medical Ethics Brief on Capacity-Based MAiD

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Document Suite

This document is one of five in the *Autonomy, Capacity, and the Right to Exit* policy suite (Bogner, 2026). The Master Policy Packet contains the complete philosophical argument, all data, and full procedural architecture. Audience-specific briefs are available for Legislative, Clinical/Medical Ethics, Academic, and Advocacy audiences.

Purpose and Audience

This brief is written for clinical ethics committees, psychiatrists, psychiatric nurses, hospital administrators, and medical associations engaged with Medical Aid in Dying policy. It addresses: (1) the clinical coherence of a capacity-based eligibility standard; (2) the assessment protocol proposed under this framework; (3) the implications for the therapeutic relationship; (4) professional liability considerations; and (5) specific clinical objections, including the depressive incapacity objection.

The Clinical Incoherence of Current Frameworks

The Informed Refusal Doctrine and Its Inconsistent Application

The foundational principle of modern medical ethics is informed consent: a competent patient may refuse any treatment, including life-sustaining treatment, on the basis of their understanding of the treatment and its outcomes, without prior utilization and without justification to the treating clinician.¹

The clinical inconsistency in current MAiD frameworks is precise: this same doctrine is not applied to the decision to seek self-directed death. A patient may refuse chemotherapy without trial; they may not, in most jurisdictions, access MAiD without demonstrating termi-

¹ This principle is codified in legislation across jurisdictions (*Health Care Consent Act*, Ontario, 1996; *Patient Self-Determination Act*, US, 1990) and affirmed in case law. See *Malette v. Shulman* (1990), 72 O.R. (2d) 417 (right to refuse emergency treatment); *Re B* [2002] 2 All ER 449 (right to refuse ventilator).

nal prognosis or externally validated suffering. The asymmetry is not clinically justified. It reflects historical and legislative contingency, not principled medical ethics.

Suffering as a Clinical Non-Category

“Unbearable suffering” is not a clinical concept. It is a legislative construct that clinical staff are asked to operationalize without any validated instrument or standard for doing so. The result is that eligibility assessment under suffering-based frameworks is: (a) subject to inter-rater unreliability; (b) sensitive to clinician bias toward or against MAiD provision; (c) dependent on the applicant’s ability to articulate subjective distress in terms intelligible to the evaluator.

Capacity, by contrast, is a clinical concept with an established literature, validated assessment frameworks, and cross-disciplinary consensus (Appelbaum & Grisso, 1988; Moye & Marson, 2007). Replacing suffering with capacity as the eligibility criterion does not lower clinical standards; it *raises* them by substituting a measurable, clinically grounded concept for a legally constructed but clinically unmoored one.

The Capacity Assessment Protocol

Operationalizing Capacity for MAiD Assessment

The Sovereignty Model applies the four-element Appelbaum-Grisso framework to MAiD:

Understanding The applicant can accurately describe what death involves (permanent, irreversible cessation), what the proposed provision entails, and what the realistic outcomes of all presented alternatives are. Understanding is assessed through structured interview, not assumption.

Appreciation The applicant understands how the general information applies to their specific situation. Appreciation does not require optimism about alternatives; it requires demonstrated recognition that the information is relevant to their case. The evaluating clinician distinguishes between a patient who says “SSRIs don’t work for anyone” (failure of understanding) and one who says “SSRIs have a documented response rate of approximately 50% for moderate depression; my history includes two adequate trials without response” (appreciation satisfied).

Reasoning The applicant can articulate a coherent account of how their understanding of their situation, the alternatives, and their values leads to their expressed preference. The reasoning need not be philosophically sophisticated; it must be internally coherent and free from gross logical errors or fixed false beliefs.

Expression of a Consistent Choice The preference is stable across the assessment period and across multiple evaluation contexts. Contextual variation (presenting differently to different evaluators) is diagnostically relevant. Persistent stability across 90 days and multiple independent evaluations satisfies this element.

The Evaluating Clinician's Role

The role of the evaluating clinician under this framework is fundamentally different from the role assigned under suffering-based frameworks.

Dimension	Protectionist Model	Sovereignty Model
Primary question	Is this person suffering enough?	Does this person understand what they are deciding?
Role	Gatekeeper / adjudicator of suffering	Assessor of comprehension and stability
Treatment history	Required; exhaustion of alternatives typical prerequisite	Irrelevant to eligibility; comprehension of alternatives is assessed
Clinical instrument	None standardized; clinician judgment on suffering	Appelbaum-Grisso framework; standardized capacity assessment
Liability orientation	Institutional (avoid death on our watch)	Patient-centered (assess authentic preference)
Therapeutic role	Custodial retention	Restorative alliance

Table 1: Comparison of clinician roles under Protectionist and Sovereignty models.

What the Clinical Interview Is Not

The clinical interview under the Sovereignty Model is not:

- A session designed to persuade the patient to choose differently. The clinician presents alternatives in sufficient specificity for genuine comprehension; they do not advocate for any alternative.
- An assessment of whether the clinician agrees with the patient's values or finds their reasoning compelling. Clinicians routinely work with patients whose values differ from their own without this constituting an eligibility barrier.
- An opportunity to require trial of interventions the patient has declined. Informed refusal of alternatives is binding. The clinical interview's role is to assess whether the refusal is informed, not to override it.

The Depressive Incapacity Objection: Clinical Analysis

This is the objection most frequently raised in clinical contexts, and it deserves careful clinical analysis rather than brief dismissal.

The Objection

Persons with treatment-resistant depression cannot satisfy the “appreciation” criterion for decisional capacity because hopelessness functions as a cognitive filter: alternatives are formally understood but experienced as inaccessible. On this account, a capacity assessment that returns “capacity present” in a person with severe depression is a false positive—the capacity criteria have been formally satisfied, but the authentic preconditions for autonomous preference are absent.

The Clinical Response

Depression does not uniformly impair capacity

The empirical literature does not support the proposition that depression, including severe depression, uniformly impairs the functional elements of decisional capacity. Appelbaum and Grisso’s foundational studies found that even significantly depressed inpatients frequently retained capacity for treatment decisions. More recent work has found that capacity impairment in depression is variable, correlates with severity, and is not reliably predicted by diagnosis alone.²

Appreciation does not require experiential access

The appreciation criterion requires that the patient understand how the information applies to their situation. It does not require that they *experience* alternatives as accessible. A patient who accurately states that SSRIs have a documented response rate for their condition, that two adequate trials without response are in their record, and that they decline further trials because the probability of benefit does not, in their assessment, justify the cost—satisfies the appreciation criterion. The criterion is epistemic, not phenomenological.

To require, additionally, that the patient *feel hope* that alternatives might succeed is to import a positive affective requirement that has no basis in the Appelbaum-Grisso framework and no precedent in clinical capacity doctrine. It also creates an impossible standard: hopelessness is a symptom of the condition being assessed; requiring its absence as a precondition for capacity makes the capacity determination circular.

The double standard problem

The depressive incapacity objection, applied consistently, would require that persons with depression satisfy a higher capacity standard than persons without depression for all medical decisions. But medicine does not currently impose this: a patient with severe depression who accepts pharmacological treatment does not have their capacity for that decision specially scrutinized. The heightened scrutiny applies only when the decision is to refuse the state-preferred intervention—revealing that the objection is not about capacity at all, but about the outcome.

² See Vollmann et al. (2003), “Competence of mentally ill patients: a comparative empirical study,” *Psychological Medicine*, 33(8): 1463–1471. Also Appelbaum & Grisso (1995), “The MacArthur Treatment Competence Study,” *Law and Human Behavior*, 19(2): 105–126.

The Clinical Protocol Response

The Sovereignty Model does not ignore the depressive incapacity concern. It addresses it procedurally. A preference that is generated by acute depressive crisis is unlikely to be stable across 90 days of assessment. A preference that survives the assessment period—including periods of relative improvement and relative worsening—and that is articulated consistently across multiple independent evaluations, has survived the procedural filter that the objection is concerned about. The process is the response.

Liability Considerations

The Current Liability Model

Current clinical practice around suicidal ideation is oriented primarily around institutional liability management: documenting that intervention occurred and that the clinician met the standard of care for suicide prevention. This orientation produces the “liability theater” documented in the existing literature: coerced safety contracts of no demonstrated efficacy, pressured hospitalization optimized for administrative rather than therapeutic outcomes, and discharge into unchanged conditions.

The clinician operating in this model is a liability-reducer first and a therapeutic ally second. This has consequences for the therapeutic relationship, for patient trust, and for clinical outcomes.

The Sovereignty Model and Liability Reform

Under the Sovereignty Model, the clinical liability framework shifts. Clinicians who conduct rigorous, documented capacity assessments following the protocol specified in this framework—multiple independent evaluations, documented alternatives presentation, independent coercion screening, 90-day assessment period—are operating within a structured and documentable standard of care. Liability exposure is reduced by procedural compliance, not by preventing death at all costs.

The conscientious objection provision is critical here: no clinician is required to participate in MAiD assessment or provision. The only obligation imposed on objectors is timely referral to a willing provider. This protects clinical autonomy while ensuring that patient access is not foreclosed by institutional objection.

The Therapeutic Consequence

The most significant clinical consequence of the Sovereignty Model is the reorientation of the therapeutic relationship. When the patient possesses an enforceable exit right, the clinician loses the coercive power of custodial retention. They cannot function as a jailer mandating biological continuity.

This constraint is not a limitation on clinical effectiveness; it is the precondition for a different kind of clinical effectiveness. The therapeutic relationship becomes genuinely collaborative. The clinician's role is to make the patient's life worth continuing—to address the conditions that are driving the preference, to connect the patient with resources they may not know exist, to work on the quality of the life rather than the fact of its continuation. This is a more honest, and ultimately more effective, clinical orientation.

Assessment Protocol Summary

Stage 1 — Decisional Capacity Assessment (multiple independent evaluators)

- Understanding: permanence of death; realistic outcomes of all alternatives
- Appreciation: application of information to own situation
- Reasoning: coherent, internally consistent account
- Expression: stable preference across assessment period
- State assessment: no acute psychosis or crisis-state distortion *currently*

Stage 2 — Process Comprehension

- Applicant understands why safeguards exist and accepts them
- Demonstrates sustained engagement across 90-day minimum period
- Failure to engage with Stage 2 is diagnostically relevant to Stage 1

Required Elements of Every Assessment

- Documented alternatives presentation with referrals offered; declinations recorded
- Independent coercion screening by unaffiliated social worker or advocate
- Financial and dependency circumstances review
- Offer of independent legal assistance
- Socioeconomic documentation where inadequate support is identified as a driver

Revocation: Absolute, unconditional, available until the moment of provision.

Clinical Objections: Summary Responses

Response: Capacity assessment in psychiatric populations is a well-established clinical practice with validated instruments (MacArthur Competence Assessment Tool; Aid to Capacity Evaluation). The proposed framework applies these instruments to a new context; it does not require new clinical methodology. The challenge of assessing capacity in complex cases is an argument

for rigorous multi-evaluator assessment, not for categorical exclusion of psychiatric populations from eligibility.

Response: The conscientious objection provision protects clinicians who decline to participate. The multi-evaluator requirement means no single clinician carries the assessment alone. The documented capacity protocol creates a professional standard of care that protects clinicians who conduct rigorous assessments, regardless of outcome. The pressure concern is addressed by process, not by maintaining a categorical prohibition.

Response: Suicide prevention is directed at crisis-state, impulsive, and untreated-pathology-driven deaths. The Sovereignty Model is addressed at a categorically different population: persons with stable, long-considered, informed preferences who have survived a 90-day assessment process. These populations do not overlap clinically. Indeed, the explicit, documented nature of MAiD provision—as distinct from unassisted suicide—may reduce rather than increase the broader social normalization concern, since it is administered within a visible, regulated, documented clinical process.

Key Clinical References: Appelbaum & Grisso (1988), *NEJM* 319(25); Appelbaum & Grisso (1995), MacArthur Treatment Competence Study, *Law and Human Behavior* 19(2); Beauchamp & Childress, *Principles of Biomedical Ethics* (8th ed., 2019); Donnelly & Campbell, *Journal of Medical Ethics* 50(3), 2024; Health Canada, *Sixth Annual Report on MAiD* (2025); Moye & Marson (2007), *J. Gerontology: Psychological Sciences*; Vollmann et al. (2003), *Psychological Medicine* 33(8).